



February 20, 2026

VIA ELECTRONIC SUBMISSION

The Honorable Robert F. Kennedy, Jr.
Secretary U.S. Department of Health and Human Services
Mary E. Switzer Building
330 C Street SW, Washington, DC 20201

Re: 2025-23641; RIN 0955-AA13 Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Dear Secretary Kennedy:

On December 23, 2025, the United States Department of Health and Human Services (HHS) published a Request for Information to accelerate the adoption and use of artificial intelligence in clinical care and ensure its safe, effective, and responsible integration into the healthcare system.¹

VieCure appreciates the opportunity to comment on the HHS Request for Information seeking input on actions to accelerate the adoption and use of AI in clinical care. VieCure is a cancer care innovation company that provides a comprehensive AI digital ecosystem to advance precision medicine in community cancer care settings.

VieCure is dedicated to advancing AI-enabled, evidence-based care delivered through our suite of digital applications for healthcare providers ("Platform"). Our Platform includes a patented clinical decision support platform ("CDS"), an integrated electronic health record system ("EHR"), and mobile applications available to community oncology providers. We strongly support a federal strategy that ensures equitable access to innovation, protects patients' timely access to guideline-concordant care, and reduces friction in adopting AI applications.

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1. U.S. Department of Health and Human Services. (2025). Request for information: Accelerating the adoption and use of artificial intelligence as part of clinical care (RIN 0955-AA13). Federal Register, 90(60108–60109).

Despite the transformative potential of AI-enabled tools to improve cancer care, two systemic barriers continue to limit equitable adoption across the settings where most Americans receive treatment.

First, there is a significant disparity in access to advanced digital infrastructure and clinical decision support across community and rural oncology practices—even though these sites deliver the majority of U.S. cancer care. Without targeted support, these providers lack the resources needed to implement, operationalize, and maintain AI-driven technologies that enable guideline-concordant, precision cancer care.

As a result, patients treated outside major academic centers face uneven access to innovative tools that could meaningfully influence outcomes. To close this gap, HHS should prioritize rural and community cancer centers in federal AI adoption programs and ensure these sites have the technical, operational, and financial support needed to implement and sustain AI solutions successfully.

Second, to sustain long-term adoption, incentives must be aligned with existing reimbursement structures and real-world oncology workflows, so that providers are encouraged and financially able to integrate AI-enabled decision-making support into daily clinical practice over time, not just during isolated initiatives. Aligning incentives to reflect the ongoing value AI delivers is essential to drive durable adoption, reduce care variability, and ensure patients benefit from consistent, evidence-based treatment regardless of geography.

HHS Should Recognize the Critical Role of AI in Equitable Care Access for Community Care Settings with Targeted Funding and Grants to Community Clinics

Community oncology is where most Americans diagnosed with cancer receive their care, and it is often more accessible, more cost-efficient, and closer to patients' homes compared with hospital/academic settings.^{2,3} Certain populations, however, continue to face obstacles accessing care, for example, according to the National Comprehensive Cancer Network, rural patients face higher cancer mortality despite lower incidence, and structural barriers. Additionally, financial barriers, transportation, digital access, and social inequalities limit equitable outcomes.³ In our experience, community centers have limited resources to adopt new therapies due to the lack of support for information at the point of care. Additionally, they lack access to sophisticated technology support teams to help them adopt new AI technologies. AI-enabled solutions, led by clinical decision and other AI-connected applications, equip clinicians with the tools required to deliver personalized medicine without manual research at the point of care and help reduce these disparities.

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2. The ASCO Post. Preferred Sites of Cancer Care: Community Oncology Practices vs Hospitals and Academic Centers. Published May 3, 2024. <https://ascopost.com/news/may-2024/preferred-sites-of-cancer-care-community-oncology-practices-vs-hospitals-and-academic-centers/> 3. Florida Cancer Specialists & Research Institute. Evaluating the Site of Service and Oncology Care Costs. April 2024. <https://flcancer.com/wp-content/uploads/2024/04/21190-FCS-SiteofService-CaseStudy-0424-FINAL.pdf>

Recommendation 1: *HHS should explicitly recognize community and rural oncology practices as a priority deployment environment for AI. HHS should direct targeted support, technical assistance, and matched grants to expand AI-enabled care-coordination capabilities, including clinical decision-making, in independent and network-based community practices, the settings most at risk of being left behind by uneven AI diffusion and utilization.*

HHS Should Partner with CMS to Create Reimbursement Pathways for AI Technologies

AI can streamline documentation, reduce administrative burden, help triage patient care options, and enhance providers' ability to make evidence-based decisions at the point of care.

Numerous AI tools exist across the oncology workflow to enhance healthcare operations that face limited resources. The value will come not only from streamlining operations but also from clinicians' ability to make the best decision for the patient when presented with both evidence and patient-driven information at the point of care.

The knowledge base of clinical options is rapidly expanding, with novel diagnostics, therapeutics, and supportive care, to the point that clinicians are becoming overwhelmed by complexity. AI tools can help reduce this complexity in a timely way, thereby reducing wait times for care and improving patient satisfaction with care and outcomes.

Currently, Ambient AI scribes are among the fastest-adopted generative AI tools in health systems. They are associated with reductions in clinician burnout and cognitive load. These benefits are especially valuable in oncology, where the burden of both decision-making and documentation can be high amid the rapid pace of new diagnostic and therapeutic innovations.^{4,5,6}

However, the high upfront and ongoing costs of AI platforms, integration, and compliance disproportionately strain community practices operating on thin margins with limited resources to support implementation and maintenance. Surveys of health systems identify financial concerns, regulatory uncertainty, and tool immaturity as top barriers to AI adoption.⁷ Without aligned incentives, advances will concentrate in large systems, widening disparities and gaps in care within community care and thereby jeopardizing the timeliness and quality of care received by the majority of US citizens diagnosed with cancer seen in the community oncology setting.

Recommendation 2: *HHS should partner with CMS to establish reimbursement pathways—within existing models such as the Enhancing Oncology Model (EOM) or other models that exist in other areas of CMS for technology adoption—that recognize the clinical and operational value of validated, clinician-in-the-loop AI. This includes allowing add-on payments or implementing a*

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4. McCarron ST, Schatz A, Hood V, et al. NCCN Policy Summit: Cancer Across Geography. *J Natl Compr Canc Netw*. 2024;22(5):284-292. <https://jnccn.org/view/journals/jnccn/22/5/article-p284.xml> ; 5. Peterson Health Technology Institute (PHTI). Leading Health Systems: AI-Powered Scribes Alleviate Clinician Burnout; Financial Impact Unclear. March 25, 2025. <https://phti.org/announcement/ai-scribes-reduce-clinician-burnout/> ; 6. Olson KD, Meeker D, Troup M, et al. Use of Ambient AI Scribes to Reduce Administrative Burden and Professional Burnout. *JAMA Netw Open*. 2025;8(10):e (online). <https://jamanetwork.com/journals/jamanetworkopen/fullarticle/2839542> ; 7. Poon EG, Lemak CH, Rojas JC, Guptill J, Classen D. Adoption of artificial intelligence in healthcare: survey of health system priorities, successes, and challenges. *J Am Med Inform Assoc*. 2025;32(7):1093-1100. <https://academic.oup.com/jamia/article/32/7/1093/8125015>

technology-enabled modifier when practices deploy AI-powered tools such as CDS, precision medicine enablement tools, or operational AI applications that measurably enhance guideline concordance, expand access to innovative therapies, or improve care delivery efficiency in community oncology settings.

Conclusion

VieCure stands ready to collaborate with providers, professionals, and governmental organizations to advance the safe, equitable adoption of AI in clinical care. As HHS considers strategies to accelerate responsible AI deployment, it is essential that federal policy reflects the realities of where most cancer patients receive treatment and the structural barriers that limit access to innovation in those settings.

- *To achieve meaningful and equitable progress, HHS should prioritize community and rural oncology practices for targeted federal support, technical assistance, and grant funding so these sites have the infrastructure and resources needed to implement and sustain AI-enabled clinical decision-making and care coordination tools.*
- *HHS should partner with CMS to establish reimbursement pathways that recognize the clinical, operational, and patient outcome value of validated, clinician-in-the-loop AI, including through add-on payments or technology-enabled modifiers within models such as the EOM.*

By clearly prioritizing community oncology and offering incentives that make AI adoption feasible for practices operating on thin margins, HHS can ensure that AI-enabled precision medicine is accessible to all patients. By aligning federal investment with the areas of greatest need and opportunity, HHS can help reduce care variability, expand access to innovative therapies, and promote guideline-concordant cancer care nationwide.

VieCure appreciates HHS's leadership and welcomes continued dialogue as these important policies advance.

Sincerely,

Michael Power

Chief Executive Officer, VieCure